

MedCore Hospitals & Healthcare Services Pvt. Ltd.

Insurance Claims SOP

Finance & Billing Department — Standard Operating Procedure

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Classification	Internal	Document Owner	Chief Financial Officer

1. Purpose

This Standard Operating Procedure (SOP) defines the steps followed by MedCore Hospitals to process insurance-linked patient episodes, from pre-authorization through final settlement or reimbursement documentation. It ensures that cashless and reimbursement claims are handled consistently across all facilities, that turnaround times are predictable for patients, and that revenue leakage arising from claim rejections or delays is minimized.

This SOP applies alongside the Hospital Billing Policy, and should be read together with it; this document focuses specifically on the interaction with insurers and Third-Party Administrators (TPAs), while the Billing Policy governs billing to the patient directly.

2. Scope

This SOP covers all patients who declare an insurance policy at the time of registration, whether the intended settlement route is cashless or reimbursement. It applies to the Insurance Desk, Billing Department, Clinical Documentation team, and any department head asked to furnish clinical justification in support of a claim.

It does not cover corporate-sponsored billing under a direct employer agreement, which is addressed in the Hospital Billing Policy, nor does it cover government scheme billing, which follows scheme-specific package rules maintained separately by the Finance Department.

3. Definitions

Term	Definition
Pre-Authorization	Approval obtained from the insurer/TPA before or shortly after admission, confirming that cashless treatment will be honored up to a specified limit.
Network Hospital	A hospital, including MedCore facilities, empanelled with an insurer's cashless network.
Initial Authorization Letter (IAL)	The insurer/TPA's first response to a pre-authorization request, which may approve, query, or reject the request.
Enhancement Request	A request to increase an already-approved authorization limit, raised when treatment cost exceeds the initial approval.
Final Authorization Letter (FAL)	The insurer/TPA's final approval issued at or after discharge, confirming the settleable amount.
Repudiation	Full or partial denial of a claim by the insurer/TPA, with reasons stated.
Non-Payable Items	Items excluded from insurance settlement under the policy terms, billed directly to the patient.

Claim Reconciliation	The process of matching amounts billed, approved, received, and outstanding for a given claim.
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4. Roles and Responsibilities

Role	Responsibility
Insurance Desk Coordinator	Verifies policy details, raises pre-authorization and enhancement requests, tracks TPA correspondence
Treating Physician / Department	Provides clinical notes, treatment plans, and justification documents needed to support the claim
Billing Executive	Prepares the itemized bill used as the basis for the claim; flags non-payable items to the patient
Insurance Desk Supervisor	Reviews queried and partially repudiated claims before escalation; approves standard clarification responses
Finance Manager	Reviews claims repudiated beyond the Insurance Desk Supervisor's authority; oversees monthly claims ageing report
Chief Financial Officer	Final authority on high-value disputed claims and on write-off of irrecoverable claim shortfalls

5. Pre-Authorization Process

Pre-authorization is the foundation of a smooth cashless claim and must be initiated as early as possible in the patient's episode, ideally before admission for planned procedures and immediately after stabilization for emergencies.

5.1 Planned Admissions

For planned admissions and elective procedures, the Insurance Desk Coordinator verifies the patient's policy status, sum insured, and any applicable waiting period at least twenty-four hours before the scheduled admission date, wherever the appointment lead time allows. The pre-authorization request is submitted with the estimated cost of treatment prepared jointly with the treating department.

5.2 Emergency Admissions

For emergency admissions, treatment begins immediately regardless of insurance status, in line with the Emergency Department SOP and the Hospital Billing Policy. The Insurance Desk Coordinator initiates the pre-authorization request within twenty-four hours of admission, or as soon as the patient's condition and family are available to provide policy details.

5.3 Required Information for Pre-Authorization

- Patient demographic details and policy number, matched against the insurer's records
- Provisional diagnosis and proposed line of treatment from the treating physician
- Estimated length of stay and estimated cost, itemized by major service category
- Prior authorization or policy exclusions applicable to the diagnosis, where known

6. Handling the Insurer or TPA Response

The insurer or TPA responds to a pre-authorization request with one of three outcomes: approval of the full requested amount, a query requesting additional information, or rejection. The Insurance Desk follows a distinct pathway for each outcome, described below.

Insurer Response	Required Action
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Full Approval (IAL issued)	Insurance Desk records the approved limit in the HMS; treatment proceeds under the cashless arrangement up to that limit.
Query Raised	Insurance Desk Coordinator obtains the requested clarification from the treating department within one working day and resubmits promptly.
Partial Approval	Treatment proceeds; the shortfall between requested and approved amount is flagged to the patient as a potential out-of-pocket liability pending enhancement.
Rejection at Pre-Authorization Stage	Patient or attendant is informed the same day; the episode is reclassified as a reimbursement case unless the patient opts to convert to self-pay.

7. Enhancement Requests During Treatment

Where the treating physician revises the treatment plan, extends the length of stay, or where actual costs are trending above the initially approved limit, the Insurance Desk Coordinator raises an enhancement request before the approved limit is exhausted, so that the patient's cashless status is not interrupted mid-treatment.

Enhancement requests are supported by updated clinical notes and a revised cost estimate. For ICU admissions, where daily deposit revision already applies under the Hospital Billing Policy, the Insurance Desk Coordinator reviews the authorization limit at the same frequency as the deposit revision to keep the two processes aligned.

8. Discharge and Final Claim Submission

8.1 Coordinating with the Discharge Process

On receipt of the discharge intimation from the treating physician, the Insurance Desk Coordinator requests the Final Authorization Letter (FAL) from the insurer or TPA in parallel with the Billing Department preparing the final itemized bill, so that discharge is not delayed solely on account of the insurance paperwork.

8.2 Assembling the Claim File

The final claim file submitted to the insurer or TPA includes the final itemized bill, discharge summary, investigation reports relevant to the diagnosis, and any implant or device invoices, assembled by the Insurance Desk Coordinator and checked against the insurer's document checklist before submission.

8.3 Non-Payable Items

Items classified as non-payable under the patient's policy, such as administrative charges, certain consumables, or attendant amenities, are identified before discharge and billed directly to the patient, consistent with Section 8 of the Hospital Billing Policy on invoice itemization.

9. Cashless Settlement

On receipt of the FAL, the Billing Department adjusts the final bill to reflect the approved cashless amount, collects any co-payment or non-payable amount from the patient, and marks the balance as receivable from the insurer or TPA. The receivable is tracked in the claims ageing report described in Section 13 until payment is received.

10. Reimbursement Claims

Where a patient chooses the reimbursement route, or where cashless approval could not be obtained, the patient settles the bill directly with the hospital in the ordinary manner. The Insurance Desk Coordinator's role shifts from claim submission to claim support: providing the patient with the documentation needed to file a reimbursement claim with their own insurer.

10.1 Documents Provided to the Patient

- Original final bill and payment receipts

- Discharge summary and relevant investigation reports
- Prescription and pharmacy bills, where issued by the hospital pharmacy
- A claim form section completed and signed by the treating physician, where the insurer requires it

MedCore Hospitals does not submit reimbursement claims to the insurer on the patient's behalf; the Insurance Desk's role is limited to ensuring the patient receives complete and accurate documentation to support their own submission.

11. Query, Deficiency, and Repudiation Handling

After a claim is submitted, the insurer or TPA may raise a deficiency query or repudiate part or all of the claim. This section describes the escalation pathway that applies once such a response is received.

11.1 Deficiency Queries

The Insurance Desk Coordinator reviews the query, obtains any additional clinical or billing documentation required from the concerned department, and resubmits the response within the timeframe specified by the insurer or TPA, and in any case within five working days of the query being received.

11.2 Partial or Full Repudiation

Where a claim is repudiated in whole or in part, the Insurance Desk Supervisor reviews the stated reason against the clinical record. If the repudiation appears to be based on incomplete information, a reconsideration request is filed with supporting documentation. If the repudiation is upheld or is based on a genuine policy exclusion, the repudiated amount is billed to the patient and the Insurance Desk Coordinator informs the patient of the reason in writing.

11.3 Escalation Thresholds

Situation	Escalation Path
Repudiated amount up to the Insurance Desk Supervisor's threshold	Insurance Desk Supervisor reviews and closes
Repudiated amount above the Supervisor's threshold, up to the Finance Manager's threshold	Finance Manager reviews jointly with the treating department
High-value or recurring repudiation pattern from a single insurer	Chief Financial Officer reviews and may raise the matter with the insurer's network relations team

12. Coordination with Billing Disputes

Where a patient disputes a charge that is also the subject of an insurance query, the Insurance Desk Coordinator and the Billing Desk coordinate their responses so that the patient receives a single, consistent explanation rather than conflicting information from the two teams. The overall dispute escalation pathway remains as described in the Hospital Billing Policy and the Patient Complaint Resolution Policy.

13. Claims Ageing and Reconciliation

The Finance Department maintains a claims ageing report listing every cashless receivable by insurer, TPA, claim status, and days outstanding. The report is reviewed weekly by the Insurance Desk Supervisor and monthly by the Finance Manager.

Days Outstanding	Required Action
0–15 days	Routine follow-up by Insurance Desk Coordinator
16–30 days	Escalation call logged with insurer/TPA; Supervisor notified

31–45 days	Formal written follow-up; Finance Manager notified
Above 45 days	Reviewed by Finance Manager for further escalation or provisioning

Claims that remain unrecovered beyond the thresholds set by the Finance Department, after all reasonable follow-up, are referred to the Chief Financial Officer for a decision on write-off, consistent with the exceptions provision of the Hospital Billing Policy.

14. Network and Empanelment Management

The Finance Department maintains the list of insurers and TPAs with which MedCore facilities are empanelled, including the agreed tariff, credit period, and any package-based rates applicable to specific procedures. Changes to empanelment status or agreed tariffs are communicated to the Insurance Desk and Billing Department before they take effect, so that pre-authorization requests are raised against current terms.

15. Records and Documentation

All correspondence with insurers and TPAs, including pre-authorization requests, queries, approvals, and repudiation letters, is retained in the patient's billing file and in the HMS insurance module for a minimum of eight years, consistent with the retention period specified in the Hospital Billing Policy and the Regulatory Compliance Manual.

16. Exceptions

Any deviation from this SOP, including bypassing the standard pre-authorization sequence for a clinically urgent reason, must be recorded by the Insurance Desk Coordinator with the reason and countersigned by the Insurance Desk Supervisor within twenty-four hours of the deviation.

17. Frequently Encountered Scenarios

Scenario	Applicable Guidance
Patient's policy has an active waiting period for the diagnosed condition.	Cashless request is likely to be rejected; patient is advised the same day and the episode proceeds as self-pay or reimbursement, per Section 6.
Insurer approves less than the estimated cost of a planned procedure.	Insurance Desk raises an enhancement request with a revised estimate before proceeding, per Section 7.
Patient is transferred from another network hospital mid-treatment.	A fresh pre-authorization is raised at MedCore referencing the prior authorization, coordinated with the Patient Transfer Protocol.
TPA requests additional documents after the patient has already been discharged.	Insurance Desk Coordinator retrieves the documents from the patient's billing and clinical file and responds within the query timeframe in Section 11.1.
Claim is repudiated on grounds that conflict with the treating physician's clinical notes.	Insurance Desk Supervisor files a reconsideration request with supporting clinical documentation, per Section 11.2.

18. Related Documents

- Hospital Billing Policy (FIN-POL-001)
- Patient Refund Policy (FIN-POL-003)
- Discount Approval Policy (FIN-POL-004)
- Patient Admission SOP (CLN-SOP-001)
- Patient Discharge SOP (CLN-SOP-002)

- Emergency Department SOP (CLN-SOP-003)
- Patient Transfer Protocol (CLN-PROT-005)
- Patient Complaint Resolution Policy (PS-POL-003)
- Regulatory Compliance Manual (LEG-MAN-005)

19. Document Control and Revision History

Version	Date	Summary of Changes
1.0	01 April 2025	Initial release of the Insurance Claims SOP
2.0	01 April 2026	Added claims ageing thresholds; clarified enhancement request process for ICU admissions; expanded repudiation escalation pathway

Document Owner: Chief Financial Officer
Approved for internal circulation within MedCore Hospitals.